Hospitalist inpatient IBD guide

Acute severe ulcerative colitis, Crohn disease admissions, supportive care, and patients already on IBD drugs. Companion to the IBD tab of the Hospitalist toolkit. 12 Nov 2026.

Draft reference, not an order set. Clinical content supplied by the toolkit author (12 Nov 2026); layout, Truelove-Witts / Oxford / Ho scoring and the app calculator written by Claude. Ho index cut-offs taken from the original paper (Ho et al. 2004) as tabulated in published reviews. Not verified for local use: check against current CAG / ECCO / ACG guidance, your GI and colorectal surgery services and pharmacy before applying to patients.

The hospitalist's job: recognise and treat ASUC, rule out infection, prevent VTE, spot Crohn complications that need surgery or drainage, and handle patients already on immunosuppressants safely. ASUC needs IV steroids, C. difficile testing, pharmacologic DVT prophylaxis and a day-3 decision on infliximab or cyclosporine rescue versus colectomy, made with GI and colorectal surgery from admission. **Treat any IBD patient on advanced therapy as immunocompromised.**

1. Acute severe ulcerative colitis (ASUC)

Score	Criteria
Truelove-Witts severe	6 or more bloody stools/day PLUS at least one of: HR > 90, temp > 37.8 °C, Hb < 105 g/L, ESR > 30 mm/h.
Oxford (Travis), day 3	Non-response = more than 8 stools/day, OR 3 to 8 stools/day with CRP > 45 mg/L. Predicts colectomy (about 85%).
Ho index, day 3	Mean stools/day over days 1 to 3: under 4 = 0; 4 to 6 = 1; over 6 to 9 = 2; over 9 = 4. Colonic dilatation > 5.5 cm = 4. Albumin < 30 g/L on admission = 1. Steroid failure: score 0 to 1 = 11%; 2 to 3 = 43%; 4 or more = 85%.

Admission

- ☐ Send C. difficile testing on everyone.
- ☐ Limited flexible sigmoidoscopy with biopsies to rule out CMV, especially if steroid-refractory.
- ☐ IV steroids: methylprednisolone 60 mg/day, or hydrocortisone 100 mg IV 3 to 4 times daily.
- ☐ VTE prophylaxis with LMWH, even with rectal bleeding (IBD flares are strongly prothrombotic).
- ☐ GI and colorectal surgery involved from admission.
- ☐ Early labs so rescue is not delayed: IGRA (QuantiFERON), HBV serologies (HBsAg, anti-HBc, anti-HBs), lipid panel and magnesium (before cyclosporine or a JAK inhibitor).
- ☐ Daily abdominal exam; low threshold for abdominal x-ray (KUB). Colon > 5.5 to 6 cm suggests toxic megacolon: call surgery urgently.

Do not

- ☐ No routine broad-spectrum antibiotics.
- ☐ No TPN for bowel rest.
- ☐ Avoid opioids, anticholinergics, antidiarrheals (loperamide) and NSAIDs: they can precipitate toxic megacolon.

Day 3 decision

- ☐ Oxford criteria: non-response = more than 8 stools/day, or 3 to 8 stools/day with CRP > 45 mg/L. Predicts colectomy.
- ☐ Non-responders: rescue with infliximab 5 mg/kg or cyclosporine; the two are equally effective.
- ☐ Infliximab is preferred because it can be continued for maintenance. With high inflammatory burden or protein-losing colopathy (low albumin), infliximab is cleared faster, so cyclosporine or a JAK inhibitor may be favoured.
- ☐ Tofacitinib is emerging as salvage therapy, but the data are observational.
- ☐ No improvement 4 to 7 days after rescue: proceed to colectomy.

Any IBD patient on advanced therapy

- ☐ Treat as immunocompromised: treat fever as a possible opportunistic infection, and hold the drug during serious infection (discuss with GI).

2. Crohn disease admissions and supportive care

Crohn disease admissions

- ☐ Early CT or MR enterography with IV contrast to separate active inflammation from obstruction, abscess or fistula.
- ☐ Intra-abdominal abscess: IV antibiotics covering GI flora (e.g. ciprofloxacin plus metronidazole). Abscess > 2 to 3 cm: percutaneous drainage. Peritonitis or uncontrolled sepsis: urgent surgery.
- ☐ Hold steroids and biologics until source control is achieved.
- ☐ Obstruction: decompress, NPO, IV steroids only if inflammation is present. Before surgery, taper prednisone to 20 mg/day or less where possible (infection risk).
- ☐ Perianal abscess or fistula: exam under anesthesia, drainage and seton, then antibiotics plus anti-TNF.

Supportive care for every IBD admission

- ☐ IV fluids, electrolytes, transfusion as needed.
- ☐ IV iron for active inflammation or Hb < 100 g/L.
- ☐ Malnutrition screen with dietitian referral; check B12 and vitamin D.
- ☐ Adequate pain control while minimising opioids.

3. Patients already on IBD drugs

Drug class	Inpatient issues
All advanced therapies	Treat fever as possible opportunistic infection; hold the drug during serious infection. Screen TB and HBV before starting a new agent.
Thiopurines	Leukopenia, pancreatitis, hepatotoxicity. Allopurinol and febuxostat raise thiopurine levels: life-threatening myelosuppression.
Methotrexate	Teratogenic; pneumonitis; cytopenias. Do not give with TMP-SMX.
Anti-TNF	TB and fungal reactivation, HBV, worsening heart failure, demyelination.
Vedolizumab, ustekinumab, IL-23 inhibitors	Low systemic infection risk; usually continue around minor procedures.
JAK inhibitors	Herpes zoster, VTE (adds to the flare risk), MACE, lipid rise.
Cyclosporine	Nephrotoxicity, hypertension, seizures with low Mg or cholesterol, PJP risk. Follow trough levels.
Steroids	More than 20 mg/day prednisone-equivalent for over 4 weeks (especially triple immunosuppression): PJP prophylaxis. Bone protection. Stress-dose steroids perioperatively if on chronic steroids.
Vaccines	Live vaccines contraindicated on biologics or immunomodulators. Recombinant zoster vaccine is fine.

Doses: see IBD medications: doses, side effects and monitoring. Which drug when: see IBD treatment flow diagram. The original text supplied numbered references [1] to [12]; the reference list was not supplied, so citations are not reproduced here.